

Henry Ford Health

## Henry Ford Health Scholarly Commons

---

Quality Expo 2025

Quality Expo

---

9-29-2025

### Project #119: Early Conversations, Better Outcomes: Advance Care Planning in Critical Care

Kristen Chasteen

Henry Ford Health, KCHASTE1@hfhs.org

Hannah Musgrove

Henry Ford Health, hmusgro1@hfhs.org

Jessica M. Haeusler

Henry Ford Health, jhaeusl1@hfhs.org

Rachel Eklund

Henry Ford Health, reklund9@hfhs.org

Sarah Beldin

Henry Ford Health, sbeldin2@hfhs.org

*See next page for additional authors*

Follow this and additional works at: <https://scholarlycommons.henryford.com/qualityexpo2025>

---

#### Recommended Citation

Chasteen, Kristen; Musgrove, Hannah; Haeusler, Jessica M.; Eklund, Rachel; Beldin, Sarah; Ronco, Veronica; Doyle, Lori; Susalla-Boyd, Kelly; Chami, Eman; Robinson, Denise M.; Villalba, Paul S.; Grafton, Gillian; Swiderek, Jennifer; Engelhardt, Amy B.; and Gupta, Arielle H., "Project #119: Early Conversations, Better Outcomes: Advance Care Planning in Critical Care" (2025). *Quality Expo 2025*. 23.

<https://scholarlycommons.henryford.com/qualityexpo2025/23>

This Book is brought to you for free and open access by the Quality Expo at Henry Ford Health Scholarly Commons. It has been accepted for inclusion in Quality Expo 2025 by an authorized administrator of Henry Ford Health Scholarly Commons.

---

## Authors

Kristen Chasteen, Hannah Musgrove, Jessica M. Haeusler, Rachel Eklund, Sarah Beldin, Veronica Ronco, Lori Doyle, Kelly Susalla-Boyd, Eman Chami, Denise M. Robinson, Paul S. Villalba, Gillian Grafton, Jennifer Swiderek, Amy B. Engelhardt, and Arielle H. Gupta



Kristen Chasteen, MD, FAAHPM, Division Head Hospice and Palliative Medicine, Palliative Care; Hannah Musgrove, MSN, APRN, AGCNS-BC, Principal Management Engineer, Quality & Safety; Jessica Haeusler, Lead Business Intelligence Developer, Clinical & Quality Analytics; Rachel Eklund, MSN, APRN, AGCNS-BC, CCRN, Clinical Nurse Specialist, Medical ICU; Sarah Beldin, MSN, APRN, ACCNS-AG, CCRN, Clinical Nurse Specialist, Cardiovascular ICU; Veronica Ronco, BSN, RN, Senior Applications Analyst, HELIOS Inpatient Expense; Lori Doyle, MBA, RRT, Director, System Tech Transformation; Kelly Susalla-Boyd, Senior Applications Analyst, HELIOS Inpatient Expense; Eman Chami, MHA, CIC, Quality Director, Quality & Safety; Denise M. Robinson, MSN, MBA, RN, NEA- BC, Interim Chief Nursing Officer and VP Patient Care Services, Patient Care Services; Paul Villalba, MD, Director Surgical Cardiovascular ICU, Department of Anesthesiology; Gillian Grafton, DO, Director Medical Cardiovascular ICU & LVAD Program, Cardiovascular Medicine; Jennifer Swiderek, MD, Medical Director Medical ICU, Pulmonary Medicine; Amy B. Engelhardt, DO, MBA, FHM, FACOI, Chief Medical Officer, Administration; & Arielle Gupta, MD, FAAS, Associate Chief Quality Officer, Administration

Henry Ford Health, Detroit, Michigan

Abstract

**Plan / Introduction:** Advance care planning (ACP) is a patient-centered process that clarifies individuals’ medical care preferences, improving alignment with their wishes and reducing emotional and healthcare burdens, yet it remains underutilized despite recommendations to initiate conversations early in hospitalization. The goal was to increase the overall percentage of documented ACP conversations, as well as the number of ACP conversations initiated within 72 hours of hospital admission in the critical care units.

**Do / Methods:** A multidisciplinary workgroup applied the Plan-Do-Study-Act framework to identify barriers, enhance ACP Navigator tools, engage Clinical Nurse Specialists (CNS), and develop educational didactics with role-play exercises for ICU providers. Training sessions were implemented across surgical, cardiovascular, and medical ICUs from August to December 2024. Dashboards were created and used to track ACP notes and palliative care consults during inpatient admissions.

**Check / Results:** When comparing pre-intervention to intervention periods, there was improvement noted in completed ACP notes and palliative care consults during the intervention period.

**Conclusions:** The initiative facilitated earlier ACP conversations, improving provider engagement, communication, and quality of care within intensive care units.

Plan / Introduction

**Problem Description:** Despite clear benefits, the frequency of ACP conversations during hospitalization remains low, even though initiation within the first 72 hours is recommended. <sup>1-3</sup>

**Available Knowledge:** ACP is a patient-centered communication process that helps individuals define their values, goals, and preferences for future medical care, including end-of-life decisions. ACP is supported by evidence showing that it improves alignment between care and patient wishes, reduces emotional burden on families, enhances communication, and avoids unnecessary interventions. <sup>4</sup>

**Rationale:** Given its impact on patient-centered care and healthcare efficiency, increasing the rate and timeliness of ACP conversations, particularly early in hospitalization, is a critical opportunity to improve outcomes and reduce costs. <sup>5</sup>

**Goals:** The goal of this quality improvement project was to increase the overall percentage of documented ACP conversations, as well as the number of ACP conversations initiated within 72 hours of hospital admission in the critical care units, by 25% by June 2025.

Figure 1: Updated Note Title

Advance Care Planning Discussion Note

Participants

Does the patient have an Advance Directive available for review?

Yes, and I have reviewed it

No/Unavailable

\*\*\*Required Question\*\*\*

Was the patient present and able to effectively participate in the Advance Care Planning Discussion?

Yes

No

If capacity is uncertain, please ensure a formal capacity assessment is documented.

Participants in Meeting (Role and Title of Family Members and Healthcare Team)

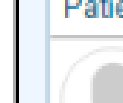
The Members of this meeting have been given the opportunity to decline Advance Care Planning conversations, and have chosen to move forward with the discussion.

Yes

No

\*\*\*Required Question\*\*\*

Figure 2: Patient List with display of ACP Note Column

| My Patient List 3 Patients                                                                                        |             |                                                         |                                             |             |               |          |          |          |          |                    |           |
|-------------------------------------------------------------------------------------------------------------------|-------------|---------------------------------------------------------|---------------------------------------------|-------------|---------------|----------|----------|----------|----------|--------------------|-----------|
| Patient                                                                                                           | Age/Gender  | Admission Info                                          | Treatment Team                              | Code Status | New Refl Flag | New Note | Com Note | Cost Ord | ACP Note | New Consult Orders | Isolation |
|  Asparagus, Charlie<br>103000010 | 60 y.o. / M | TRN IP 1A Pool01/TRN I B555<br>Microscopic gastrointest | Hunter Timely<br>- Admitting<br>Hunter Time | Full Code   | !             |          |          |          | 22 h     |                    |           |
| Inpatient                                                                                                         |             |                                                         |                                             |             |               |          |          |          |          |                    |           |

Do / Methods

The team included representatives from a range of departments and specialties, including physicians, Clinical Nurse Specialists, advanced practice providers, Information Technology (IT) specialists, and administrative leadership. The initiative was guided by the Plan-Do-Check-Act framework and the SQUIRE 2.0 guidelines. <sup>6</sup>

**Interventions:**

- A significant barrier identified was the ACP documentation note titled “End of Life Discussion,” which was perceived as inappropriate and potentially discouraged early ACP conversations. The team worked to rename the note: Advance Care Planning Discussion Note (Figure 1).
- The ACP Navigator was enhanced to make it easier to determine when the last ACP note was completed. A new column was added to the patient list view, providing a quick reference for the care team to see the most recent ACP documentation (Figure 2).
- ACP didactic sessions were created and implemented to train attending physicians, fellows, advanced practice providers, and residents. Didactic sessions were held monthly. Implementation was staggered across the surgical, cardiovascular, and medical intensive care units (ICU) between August and December 2024.
- CNS engaged teams during rounds when ACP notes weren’t completed.

**Study of the Interventions:** To assess the impact of ACP, the pre-intervention period (January-June 2024) was compared to the intervention period (January-June 2025).

**Measures:**

- ACP note completion rate: Tracked by dividing the number of patients with an ACP note on the admission unit by the total number of patients whose first inpatient unit was that department.
- ACP note within 72 hours: Number of ACP notes completed within 72 hours of hospital admission.
- Palliative care consult penetration rate: Calculated as the percentage of palliative care consults completed out of the total number of discharged patients per hospital.

**Analysis:** Custom dashboards tracked trends in ACP documentation and palliative care consults. Contact Serial Numbers (CSNs) were used to accurately identify patient encounters. Both updated and historical ACP note types were extracted to ensure accurate trend tracking. An observed increase during the intervention period was considered a key indicator of success.

Table 1: Percent Patients with ACP Note on Admission to Unit, Number of ACP Notes Completed within 72 Hours on the Admission Unit, & Palliative Care Consults

| Unit Type                           | January-June 2024 | January-June 2025 | Percent Change |
|-------------------------------------|-------------------|-------------------|----------------|
| Surgical ICU                        |                   |                   |                |
| % Patients with ACP Note            | 10%               | 21%               | 110% Increase  |
| # ACP notes withing 72 hours        | 60                | 121               | 102% Increase  |
| Palliative Care Consult Penetration | 11%               | 11%               | No Change      |
| Cardiovascular ICU                  |                   |                   |                |
| % Patients with ACP Note            | 17%               | 37%               | 118% Increase  |
| # ACP notes withing 72 hours        | 51                | 172               | 237% Increase  |
| Palliative Care Consult Penetration | 20%               | 20%               | No Change      |
| Medical ICU                         |                   |                   |                |
| % Patients with ACP Note            | 23%               | 33%               | 44% Increase   |
| # ACP notes withing 72 hours        | 218               | 305               | 40% Increase   |
| Palliative Care Consult Penetration | 15%               | 20%               | 33% Increase   |
| ICUs Combined                       |                   |                   |                |
| % Patients with ACP Note            | 18%               | 30%               | 67% Increase   |
| # ACP notes withing 72 hours        | 329               | 598               | 82% Increase   |
| Palliative Care Consult Penetration | 13%               | 16%               | 23% Increase   |

Check / Results

**Project Modifications:** Multiple modifications were made throughout the project’s implementation. When the ACP column was first launched, it did not correctly calculate note time in the production environment. This required updates to the build to ensure proper functionality in live use. During the development of the ACP dashboard, several improvements were made to ensure accurate data collection. ACP didactic sessions were presented to the Patient and Family Advisory Committee, and feedback from the committee led to revisions in the role-playing exercise. For each unit where the project was implemented, the didactic content was further reviewed and tailored to ensure it was relevant and appropriate for the specific audience. Materials were customized to meet the unique needs of each group. Although ACP didactic sessions were scheduled monthly, additional opportunities to engage other providers were identified. As a result, supplemental sessions were arranged for advanced practice providers and fellows to expand the project’s reach and impact.

**Results:** When comparing periods, overall, there was a 67% improvement observed in the percent of patients with an ACP note on admission to the unit. An 82% increase was observed in the number of ACP notes completed within the first 72 hours on the admission unit across ICUs. A slight increase in palliative care consults was also noted during the intervention period (Table 1).

Act / Discussion

**Key Findings & Interpretation:** The implementation of electronic medical record enhancements, routine ACP didactics, and CNSs engaging teams during rounds has resulted in improved documentation of ACP conversations and facilitated these conversations occurring earlier during hospital admissions. Key contributors to these improvements include setting clear expectations, streamlining documentation processes, and making it easier to identify the most recent ACP note. These factors collectively support more timely and consistent ACP communication.

**Spread:** The multidisciplinary workgroup is actively working on spread throughout Henry Ford Hospital. Two poster presentations highlighting this work were delivered at the 2025 American College of Surgeons Quality and Safety Conference, further promoting awareness and potential adoption.

**Conclusion:** This initiative supported earlier, more effective ACP conversations, leading to improved communication, increased provider engagement, and enhanced quality of care in ICUs.

Acknowledgements

The project team acknowledges the physician champions and nurse leaders on surgical, cardiovascular, and medical ICUs for their leadership, and support throughout the implementation of the initiative.

References

- Bekker, Y. A. C., Suntuens, A. F., Engels, Y., Schers, H., Westert, G. P., & Groenewoud, A. S. (2022). Advance care planning in primary care: A retrospective medical record study among patients with different illness trajectories. *BMC Palliative Care*, 21(1), 21. <https://doi.org/10.1186/s12904-022-00907-6>
- Levoy, K., Salani, D. A., & Buck, H. (2019). A systematic review and gap analysis of advance care planning intervention components and outcomes among cancer patients using the Transtheoretical Model of Health Behavior Change. *Journal of Pain and Symptom Management*, 57(1), 118–139.e6. <https://doi.org/10.1016/j.jpainsymman.2018.09.006>
- Hwang, F., Son, J., Ensor, K., Goulet, N., Brown, A., Victory, J., & Tyrie, L. (2023). Initiating advance care planning at admission: A brief intervention to increase goals of care discussions in geriatric trauma patients in an urban level I trauma center. *Trauma Surgery & Acute Care Open*, 8, e001058. <https://doi.org/10.1136/tsaco-2022-001058>
- Sedini, C., Biotto, M., Crespi Bel'skij, L. M., Moroni Grandini, R. E., & Cesari, M. (2022). Advance care planning and advance directives: An overview of the main critical issues. *Aging Clinical and Experimental Research*, 34(2), 325–330. <https://doi.org/10.1007/s40520-021-02001-y>
- Morrison, R. S., Meier, D. E., & Arnold, R. M. (2021). *What's wrong with advance care planning?* JAMA, 326(16), 1575–1576. <https://doi.org/10.1001/jama.2021.16430>
- Ogrinc, G., Davies, L., Goodman, D., Batalden, P., Davidoff, F., & Stevens, D. (2015). *SQUIRE 2.0 (Standards for Quality Improvement Reporting Excellence): Revised publication guidelines from a detailed consensus process.* American Journal of Medical Quality, 30(6), 543–549. <https://doi.org/10.1177/1062860615605176>